

Zulekha Hospital LLC - [SHARJAH] SHARJAH

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P.O. Box: 48577, DUBAI, U.A.E.

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ص ب: ٤٨٥٧٧، دبي، إ.ع.م.

هاتف: ٠٤ ٢٦٧ ٨٨٦٦ - فاكس: ٠٤ ٢٦٧ ٨٨٥٥

ص ب: ٤٥٧، الشارقة، إ.ع.م.

هاتف: ٠٦ ٥٠٦ ٠٠٠٠ - فاكس: ٠٦ ٥٦٥ ٦٦٩٩

مستشفى زليخة
لأن صحتك غاليةZulekha Hospital
Your Health Matters

Discharge Summary

Page No.:1

Allergy : None

Alerts : Coronary Artery Disease, Triple Vessel Disease.

Patient Name	DEBROY ARNAB	Adm Date	08/06/17	Discharge Date	09/06/2017	Age&Sex	48Y5M8D, MALE
ZH NO	ZHS0107220	IP NO	IP00102167	Bed No	ICU 6	PhoneNo	5559219 050-9848630
Specialist	ANIL BANSAL	Nationality	INDIAN	Ward Name	WARD1	Specialty	CARDIOLOGY

Reason For Admission :

48 years old gentleman who is known case of hypertension, Diabetes mellitus, dyslipidemia with CAD and triple vessel disease underwent attempted PCI to LAD (CTO) which didn't succeed and the procedures was abundant. Plan - CABG.

Significant Findings :

Blood pressure: 130/80 mmHg, Pulse rate: 70/min, SP02: 99% in Room air,
Temperature: 37°C, Respiratory rate: 16/min.
CVS - Respiratory examination - unremarkable.

Final Diagnosis :

CODE (ICD: I25.119) Atherosclerotic heart disease of native coronary artery with unspecified angina pectoris
CODE (ICD: I10) Essential (primary) hypertension
CODE (ICD: E78.5) Dyslipidemia, unspecified
CODE (ICD E11.9) Type 2 diabetes mellitus without complications

Surgery or Procedure Performed :

Attempted PCI to LAD - CTO.

Operation Notes/Procedure Details :

INDICATION : HYPERTENSION,DIABETES MELLITUS, TMT POSITIVE.
ACCESS : RIGHT FEMORAL ARTERY AND LEFT FEMORAL ARTERY.
PROCEDURE :
LMCA CANNULATED WITH 7F EBU 3.5cm GUIDING CATHETER AND MID LAD TOTAL OCCLUSION
TRIED TO CROSS WITH 0.014 FIELDER XT WIRE, 0.014 GAIA SECOND WIRE, 0.014 GAIA
THIRD WIRE AND 0.014 MIRACLE 6 WIRE WITH THE SUPPORT OF MICROCATETER AND WITH
RETROGRADE INJECTIONS FROM RCA .THE WIRE WAS TRACKED SUB INTIMALLY, REPETEDLY
TRIED WITH WIRE AND MICRO CATHETER TO RE ENTER THE TRUE LUMEN BUT FAILED. SO
PROCEDURE ABANDONED. FINAL CHECK ANGIOGRAM SHOWED NO EVIDENCE OF COMPLICATION
WITH PERSISTENT CTO IN LAD AND RETROGARDE COLLATERALS FROM RCA /PDA.

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ص ب: ٤٥٧، الشارقة، إ.ع.م.

هاتف: ٥٠٦٠٠٠٠، فاكس: ٥٦٥٦٦٩٩، إ.ع.م.

Discharge Summary

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HARDWARES : 7F EBU 3.5cm GUIDING CATHETER,
6F JR3.5cm DIAGNOSTIC CATHETER,
0.014 FIELDER XT WIRE,
0.014 GAIA SECOND WIRE,
0.014 GAIA THIRD WIRE,
0.014 MIRACLE 6 WIRE
CORSAIR MICROCATHETER 150CM.
MEDICATION : 5000 IU HEPARIN.
CONTRAST : 250ml ISO OSMOLAR OMNIPAQUE CONTRAST.
IMPRESSION : ATTEMPTED PCI IN LAD (CTO) .
PLAN : CABG

Medication & Treatment :

Pre cardiac cath patient admitted to ICU and received - Normal saline 150mls/hour,
Plavix 300 mg per oral stat, Aspirin 100 mg per oral stat,
Pan 40 mg IV Stat, Atocor 80 mg per oral stat.
Post cardiac cath patient previous medications was resumed and IV Normal Saline
100 mls/hour was given for 8 hours.
Patient stay in ICU remain free of complication.

Investigation Done :

Investigation reports enclosed.

Patient Condition During Discharge :

Clinically and hemodynamically stable.
Bilateral femoral access sites are normal.

Discharge Medications :

TABLET ASPIRIN 100 MG 1-0-0 FOR 2 MONTH
TABLET PLAVIX 75 MG 1-0-0 FOR 2 MONTH
TABLET ATORCOR 80 MG 0-0-1 FOR 2 MONTH
TABLET IMDUR 60 MG 1-1-0 FOR 2 MONTH
TABLET NEBIT 5 MG 1-0-0 FOR 2 MONTH
TABLET VASTAREL MR 35 1-0-1 FOR 2 MONTH
TABLET PAN 40 MG 1-0-0 FOR 2 MONTH
TABLET NATRILIX SR 1.5 MG 0-1-0 FOR 2 MONTH
TABLET NORVASC 5 MG 1-0-0 FOR 2 MONTH
TABLET RANOLAZINE 500 MG 1-0-1 FOR 2 MONTH
TABLET GALVUS MET (50/1000) 1-0-1 FOR 2 MONTH

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Advice & Follow Up Instructions :

- Avoid climbing stair and squatting for 7 days.
- Strict adhesion to medication.
- Consultation with Cardiac surgeon for CABG (urgent).
- To be seen on June 17, 2017 in Dr. Anil's OPD.
- Contact Emergency Department - Zulekha Hospital Sharjah in case of chest pain, shortness of breath, dizziness, palpitation.

Contact Person in case of emergency :

Contact Zulekha Hospital Cardiology Dept. / Emergency Department 06 -5060171 / 06-5060172 / 06-5060000

Next Appointment Date & Time :

17/06/2017 17:00

ANIL BANSAL
 Specialist: CARDIOLOGY
 Dr. Sohail Inayat

Acknowledgment Details

Received the Discharge Summary, Investigation Reports & Discharge Medicine from the ward

Name & Sign (Rec): _____

Relationship: _____

Date & Time: _____

Given by(Name & Stamp): _____